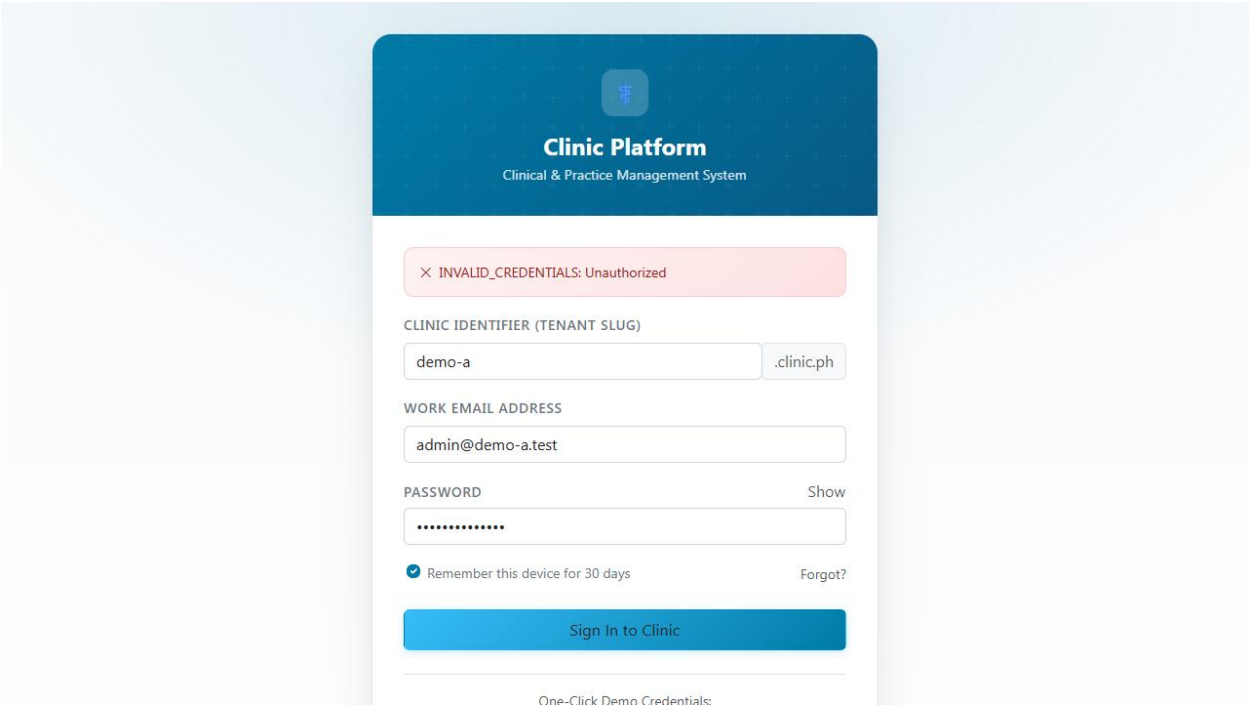
Clinic Platform — User Manual

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1. Getting Started

1.1 Login

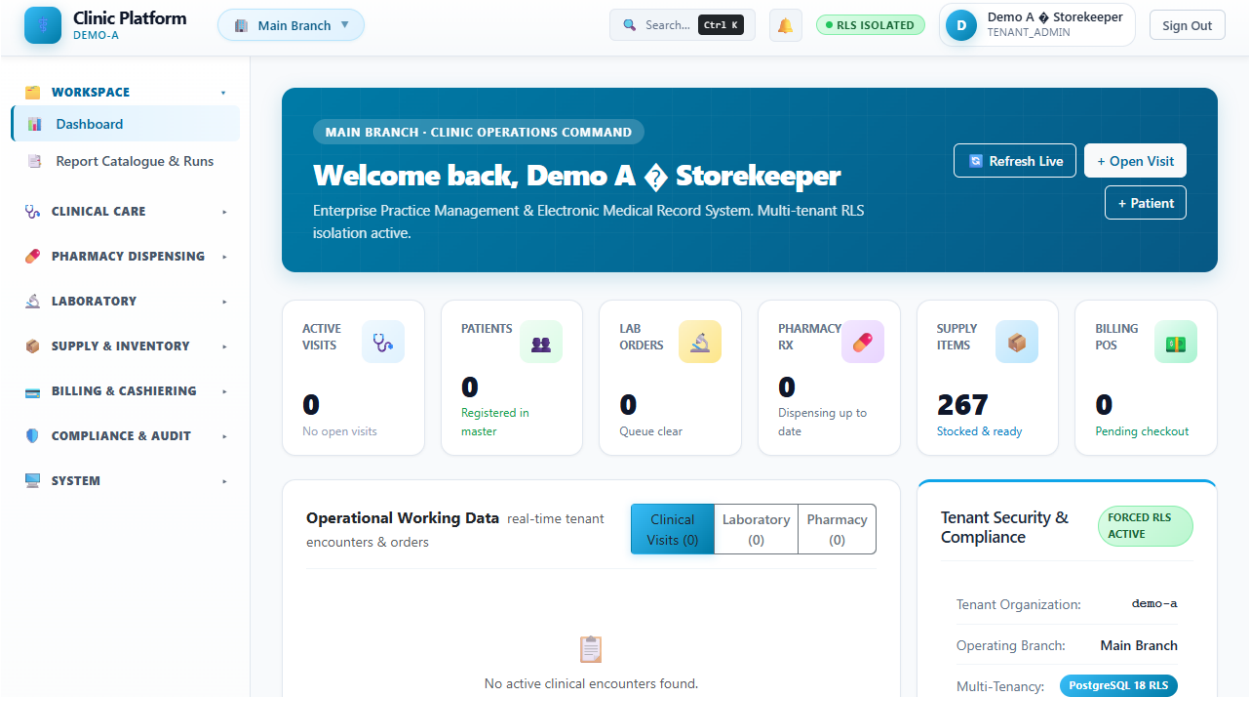


To log in, enter your tenant slug (e.g. demo-a), your email address, and your password, then click "Sign In to Clinic". The tenant slug identifies your organization — each tenant is fully isolated with its own data, users, and settings.

1.2 Navigation

The left sidebar contains the main navigation menu, organized by module: Clinical Care, Laboratory, Pharmacy, Supply & Inventory, Billing, Compliance, and System (admin). Each module expands to show its screens. The top bar shows the current branch, a global search (Ctrl+K), notifications, and the user menu.

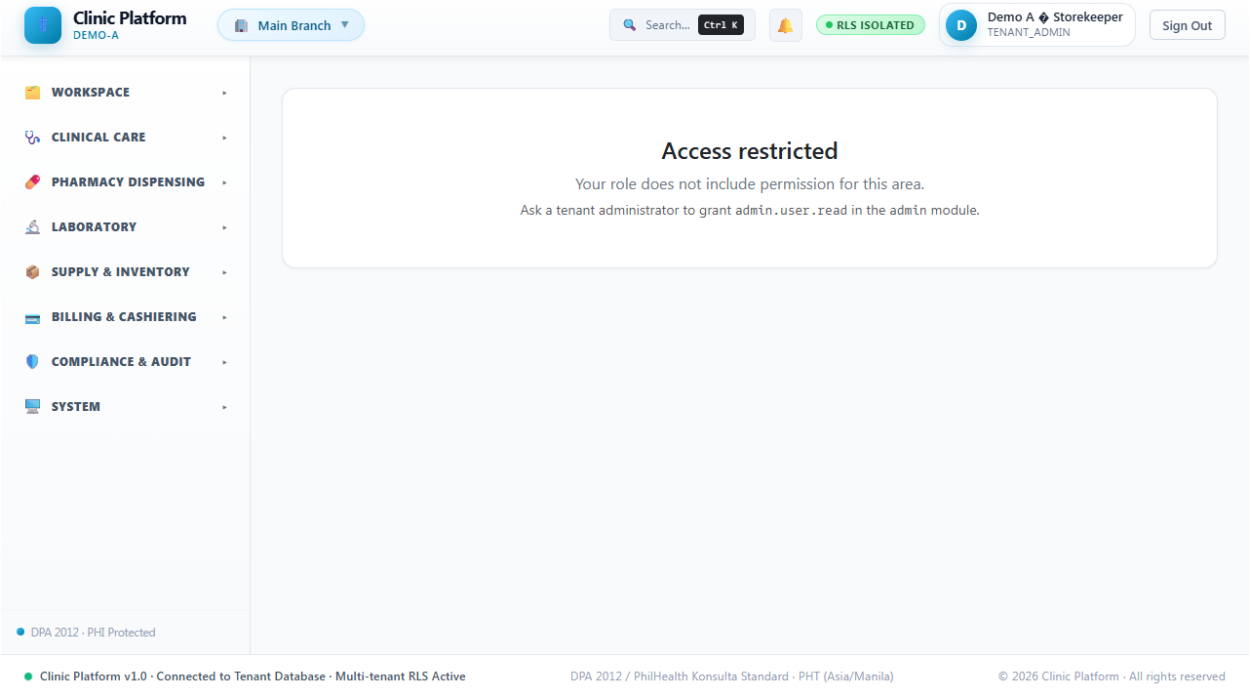
1.3 Dashboard



The dashboard is your home screen after login. It shows a summary of today's activity: appointments scheduled, pending lab orders, prescriptions awaiting verification, requisitions in progress, and billing totals.

2. Tenant Administration

2.1 Users



The Users screen (System > Users) lets tenant administrators create, update, deactivate, and reset passwords for all user accounts. Each user has an email, name, role assignment(s), and status (active/inactive). To create a user, click "New User", fill in the details, assign roles (see §2.2), and save. User accounts can be deactivated but not deleted — this preserves the audit trail.

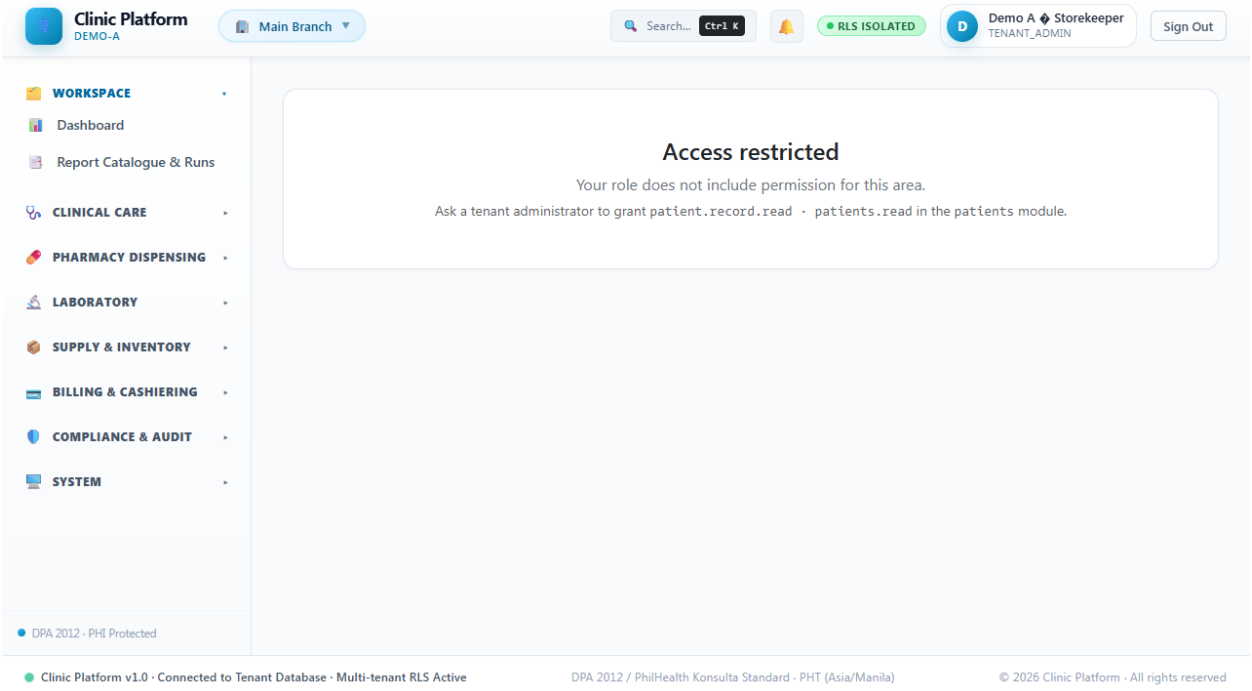
2.2 Roles & Permissions

Roles define what each user can see and do. The platform enforces segregation of duties (SoD) through role design. Key role groups include: Clinical (Doctor, Nurse, Encoder), Pharmacy (Pharmacist, Pharmacy Technician), Laboratory (Lab Technologist, Lab Verifier), Supply (Storekeeper, Supply Approver, Inventory Controller), Billing (Cashier, Billing Clerk, Branch Manager), and System (Tenant Admin, Superadmin). Roles are managed in System > Roles. To assign a role, edit the user and select from available roles. Note that SoD prevents assigning conflicting roles to the same user (e.g. a Lab Technologist cannot also be a Lab Verifier).

2.3 Branches

Branches represent physical locations of your organization. Each branch has its own stock locations, service units, and operating hours. Users can be assigned to one or more branches. The branch switcher in the top bar changes the active branch context — all data shown is scoped to the selected branch.

3. Patients



3.1 Patient Registration (CRUD)

The Patients screen is the master patient index (Clinical > Patients). To register a new patient, click "New Patient" and fill in demographics (name, date of birth, sex, contact information, address). The system generates a unique Medical Record Number (MRN) automatically. You can also search for existing patients using the global search or the filter bar. To update a patient record, open the patient profile and click "Edit". To deactivate a patient (e.g. if the record was created in error), click "Deactivate" — the record is preserved but marked inactive and no longer appears in active searches.

Patient documents (consent forms, lab results, imaging reports, referrals) can be uploaded and attached to a patient record. Each document is stored with access controls — only users with patient.document.read permission can view. Documents can be viewed inline or downloaded. To upload, open the patient profile, go to the Documents tab, and click "Upload Document". Select the file, choose the document type, and add optional notes. The document is encrypted at rest and a PHI access log entry is created.

4. Clinical

4.1 Appointments & Scheduling

The Appointments screen (Clinical > Appointments) shows a calendar view of all scheduled appointments. To book an appointment, click on a time slot or click "New Appointment", select the patient, practitioner, service unit, date, and time. The system checks for scheduling conflicts and sends a confirmation. Appointments can be rescheduled (drag-and-

drop on the calendar) or cancelled (with a reason code). The status workflow is: SCHEDULED → CHECKED_IN → IN_PROGRESS → COMPLETED (or CANCELLED).

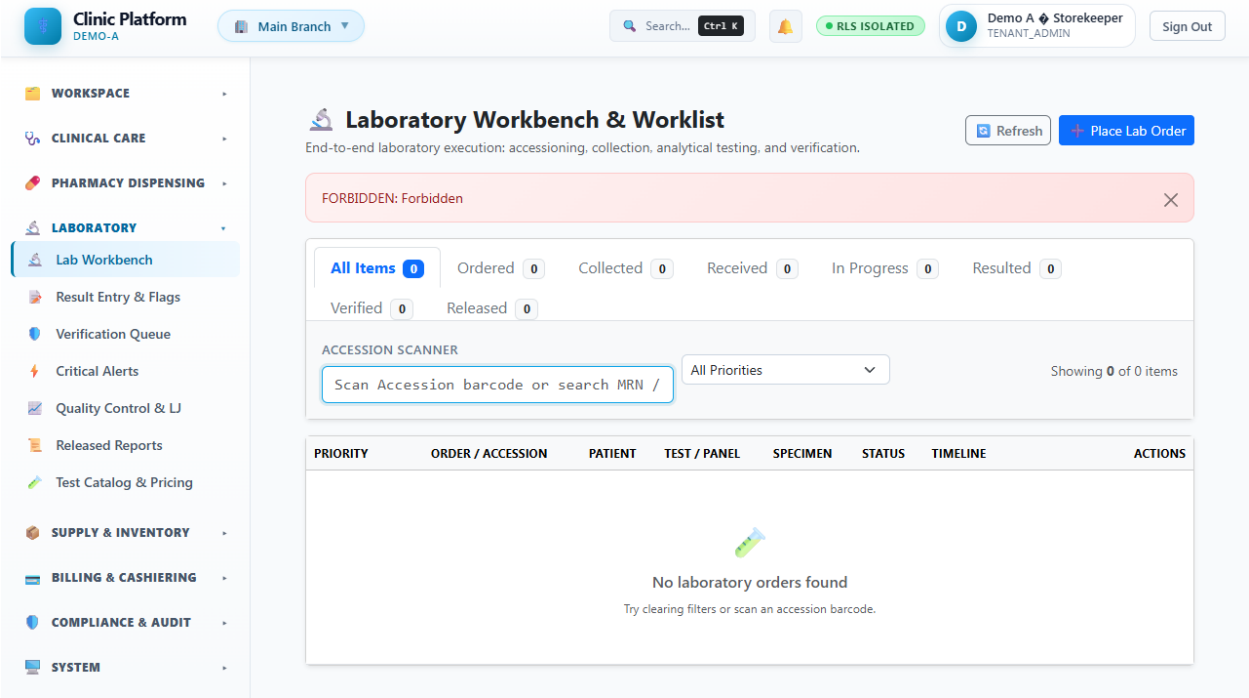
4.2 Visits & Clinical Notes

A visit is created when a patient is checked in for a consultation. The visit record captures the chief complaint, vitals (blood pressure, heart rate, temperature, weight, height, SpO2), and the clinical notes entered by the doctor. Clinical notes use a structured template — the doctor enters subjective findings, objective measurements, assessment, and plan (SOAP format). Notes can be saved as drafts and signed when complete. Signed notes are locked and can only be amended (with a reason) — never edited directly.

4.3 Prescriptions

Prescriptions are created during a visit by a doctor. Each prescription contains one or more line items (drug, dose, frequency, duration, quantity). The system checks for drug interactions, allergies, and duplicate therapy before the prescription can be signed. Signed prescriptions appear in the pharmacy queue for verification and dispensing. Prescriptions can be cancelled (with reason) but cannot be edited after signing — corrections require an amendment.

5. Laboratory



5.1 Test Catalog & Pricing

The Test Catalog (Laboratory > Setup > Test Catalog) defines all laboratory tests offered by the clinic. Each test has a code (LOINC), name, section (e.g. Hematology, Chemistry, Microbiology), specimen type, container/tube color, reference ranges, turnaround time, and

price. Tests can be created, updated, and deactivated by users with lab.config permissions. The catalog drives ordering, specimen collection, and result entry. To add a new test, click "New Test" and fill in the form, including analytes and their reference ranges.

5.2 Lab Orders & Worklist

Lab orders are created by doctors during a visit or by encoders from the order screen. Each order contains one or more test items. The worklist (Laboratory > Worklist) shows all orders with their current status: ORDERED (pending collection), COLLECTED (specimen received), ENTERED (results entered), VERIFIED (results verified), RELEASED (official report generated), CANCELLED. To collect a specimen, select the order item and click "Collect Specimen". The system generates an accession number and records the collection time and the collector.

5.3 Result Entry & Verification (AC-17)

The lab technologist enters results for each analyte on the result entry screen (Laboratory > Result Entry). The system automatically flags values that are critical (LL or HH) or abnormal compared to reference ranges. After results are entered, a different user with the lab_verifier role must verify each result on the Verification Queue (Laboratory > Verification). This is a mandatory two-person check (AC-17 — Segregation of Duties): the person who entered the result cannot verify it. The verifier can accept, reject (returns to entry), or amend results with a reason.

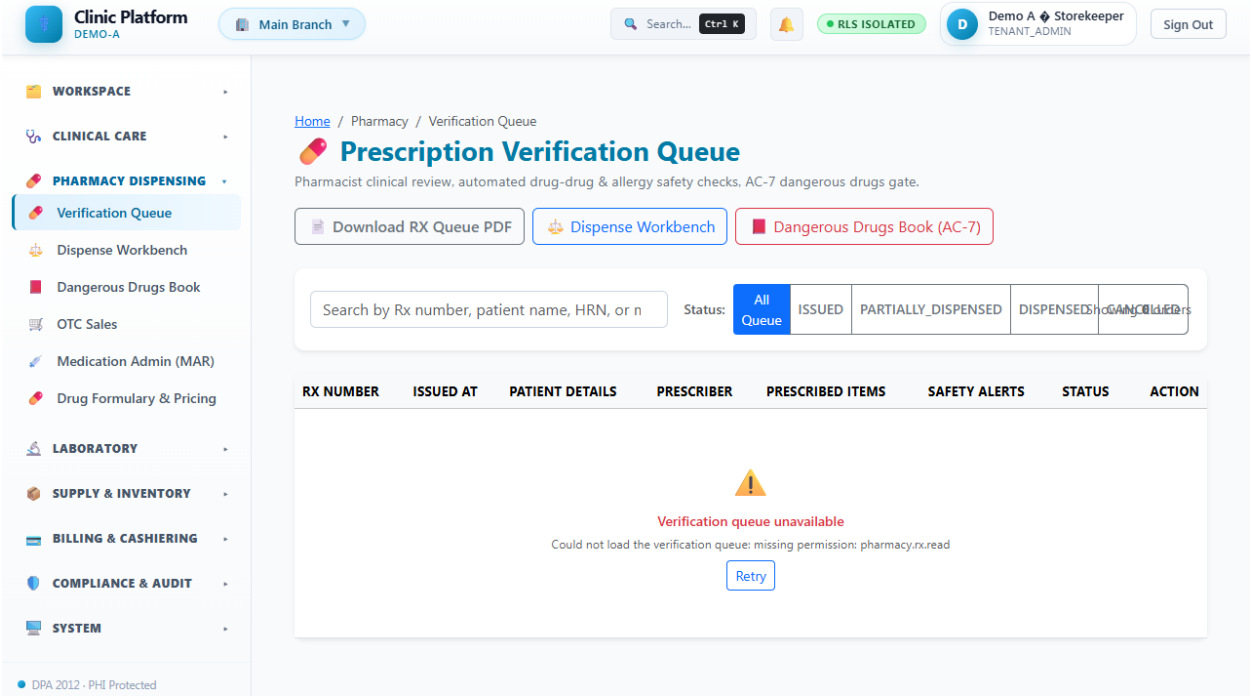
5.4 Critical Values & Alerts

When a result is flagged as critical (LL or HH), the system creates a critical alert on the Critical Alerts screen (Laboratory > Critical Alerts). The alert must be acknowledged by a qualified user, and the acknowledgment is logged with the user, timestamp, and any action taken. Critical alerts cannot be dismissed without acknowledgment — this ensures urgent results are communicated to the attending physician promptly.

5.5 Released Reports

Once all results are verified, the lab verifier releases the official report (Laboratory > Released Reports). The Released Reports screen shows all finalized reports with patient name, MRN, order number, test name, and release date. Reports can be printed, downloaded as PDF, or shared to the patient portal. The "Collect All" function on the worklist marks all uncollected specimens as collected at once — use this carefully as it affects tracking.

6. Pharmacy



6.1 Drug Master

The Drug Master (Pharmacy > Setup > Drug Master) is the catalog of all medications available in the pharmacy. Each drug has a generic name, brand name, strength, dosage form, unit of measure, category, controlled substance flag, reorder level, and price. Drugs can be created, updated, and deactivated. The drug master drives prescribing, dispensing, and stock management. To add a new drug, click "New Drug" and fill in all required fields including the controlled substance classification if applicable.

6.2 Prescription Verification

When a prescription is signed by a doctor, it appears in the Pharmacy Queue (Pharmacy > Queue). The pharmacist reviews each prescription for correctness, drug interactions, allergies, duplicate therapy, and appropriateness. The verification screen shows the patient's allergy history, current medications, and the prescribed items. To verify, click on the prescription, review the details, and click "Verify & Dispense" or "Reject" (with a reason). Rejected prescriptions are returned to the doctor for review.

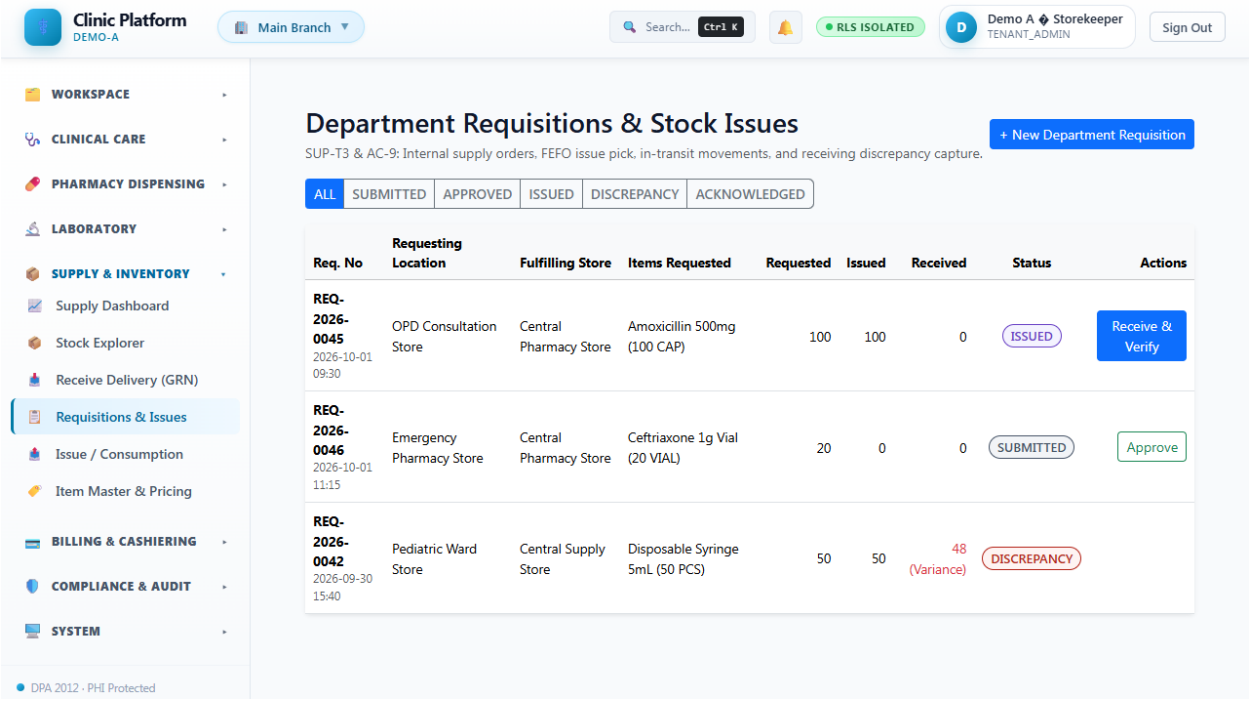
6.3 Dispensing (FEFO)

Dispensing is the process of allocating stock to a verified prescription. The system uses FEFO (First-Expired, First-Out) to select the batch with the earliest expiry date, minimizing waste. The pharmacist selects the verified prescription, reviews the suggested batch allocation, and clicks "Dispense". The system prints a dispensing label, updates stock balances, and generates a dispensing record. The patient receives the medication, and an audit trail entry is created for this dispense.

6.4 Controlled Drugs Register

Controlled substances (dangerous drugs under RA 9165) require special handling. The system maintains a controlled drug register (Pharmacy > Reports > Controlled Drug Register) that records every receipt, dispensing, usage, and disposal with mandatory witness signatures. Only users with the pharmacy.controlled permission can access the register. Each entry is signed by two authorized individuals (dispenser and witness). The register can be exported for BFP (Bureau of Firearms and Protection) reporting and generates a monthly physical inventory count.

7. Central Supply



7.1 Item Master & Locations

The Item Master (Supply > Setup > Item Master) is the catalog of all supply items (medical supplies, consumables, reagents, office supplies). Each item has a code, name, category, unit of measure, reorder level, and par stock. Locations define the physical storage hierarchy: tenant → branch → store → bin. Each location has a name, type (central store, department, pharmacy, lab), and parent location. Items can be assigned to specific locations. To add an item, click "New Item" and fill in all required fields. To manage locations, go to Supply > Setup > Locations.

7.2 Department Requisitions

Department requisitions are internal requests from clinical departments (OPD, Emergency, Ward) to the central store for supplies (Supply > Requisitions). The requesting department creates a requisition with line items (item, quantity needed), then submits it. The supply

approver reviews and approves or rejects it. Once approved, the storekeeper picks the items from stock and issues them. The requesting department receives and verifies the items, acknowledging any discrepancies. The requisition status workflow is: DRAFT → SUBMITTED → APPROVED → PICKED → ISSUED → RECEIVED → CLOSED. A storekeeper role can view all requisitions on the dashboard.

7.3 Purchase Orders & GRN

Purchase Orders (Supply > Purchase Orders) are external orders to suppliers for stock replenishment. The purchaser creates a PO with line items (supplier item, quantity, unit price), then submits it. The supply approver reviews and approves the PO. Once approved, the PO can be sent to the supplier (via email or integration). When goods arrive, the storekeeper creates a Goods Receipt Note (GRN) by matching the delivery against the PO, recording quantities, batch numbers, and expiry dates. The GRN is posted to update stock balances.

7.4 Invoice Matching (3-Way Match)

When the supplier invoice arrives, the inventory controller performs a three-way match (Supply > Invoice Matching): the invoice is matched against the purchase order (what was ordered) and the GRN (what was received). The system verifies that quantities and prices agree across all three documents. Only when the match is successful can the invoice be approved for payment. This prevents overpayment for goods not ordered or not received.

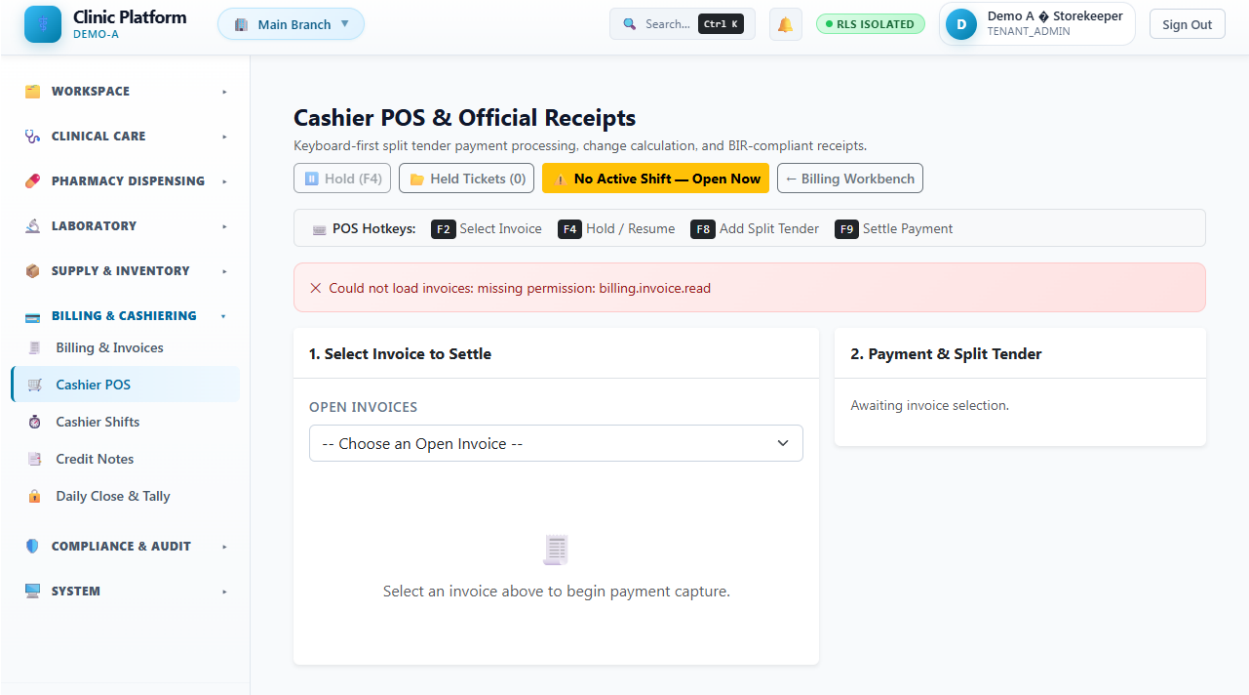
7.5 Stock Adjustments

Stock adjustments (Supply > Adjustments) correct discrepancies between physical stock and system stock. The storekeeper creates an adjustment (increase or decrease) with a reason (damage, expiry, theft, correction). The adjustment is submitted to the supply approver for approval, and once approved, the inventory controller posts the adjustment, which updates the stock balance and creates an audit trail entry. Adjustments are append-only ledger entries — the system never edits existing stock transactions directly.

7.6 Cycle Counts

Cycle counts (Supply > Cycle Counts) are periodic physical stock counts to verify system accuracy. The storekeeper creates a count sheet for a location or item category, enters the physical quantities, and submits it. The system compares the physical count to the system balance and flags variances. The supply approver reviews and approves the count, and the inventory controller posts any resulting adjustments. The Stock Explorer (Supply > Stock Explorer) provides real-time visibility into stock levels, batch details, and expiry dates.

8. Billing & Cashiering



8.1 Charges & Invoices

Charges are created when services are rendered (Billing > Charges). Each charge links to a patient, a service or item, a quantity, and a unit price. Charges accumulate on the patient's account until an invoice is generated. The cashier can create an invoice from outstanding charges by going to the POS screen and clicking "Generate Invoice". The invoice captures all charges for the visit. Invoices can be voided (with reason) but not edited after posting. To view a patient's billing history, open the patient profile and go to the Billing tab.

8.2 Payments & POS (Point of Sale)

The Point of Sale (Billing > POS) is the cashier's primary work surface. It shows the patient's outstanding balance and allows the cashier to select a payment method (cash, card, insurance, etc.), enter the tendered amount, and calculate change. Once payment is confirmed, the system generates an Official Receipt (OR) with a unique receipt number, updates the invoice status to PAID, and prints the receipt. The POS supports split payments (e.g. partial cash + partial card). To start a transaction, search for the patient, select the invoice, enter the payment details, and click "Complete Payment".

8.3 Cashier Shifts

Cashier shifts (Billing > Shifts) track the cash drawer across a working period. The cashier opens a shift with a starting float amount, processes payments throughout the day, and closes the shift by counting the physical cash and comparing it to the system total. Any overage or shortage is recorded with a reason. The shift close screen shows expected vs

actual totals by payment method. To open a shift, click "Open Shift" and enter the float amount. To close, click "Close Shift" and enter the counted amounts.

8.4 Credit Notes

Credit notes (Billing > Credit Notes) are issued when a patient is overpaid or a service is cancelled after payment. The cashier creates a credit note against the original invoice, specifying the amount and reason. The credit note is submitted for approval, and once approved, it can be applied to future invoices or refunded. Credit notes maintain a full audit trail linking back to the original payment.

9. Compliance & Audit

9.1 Audit Trail

The Audit Trail (Compliance > Audit Trail) shows a chronological log of every action taken in the system — who did what, when, and from which IP address. It covers patient record access, clinical note creation and signing, prescription changes, result entry and verification, stock movements, payment posting, and administrative actions. The audit trail is immutable and cannot be edited or deleted. It can be filtered by user, date range, module, or action type, and exported for regulatory review.

9.2 PHI Access Log

The PHI Access Log (Compliance > PHI Access) is a specialized view of the audit trail that shows every access to patient health information. Under the Data Privacy Act (RA 10173), clinics must be able to demonstrate who accessed patient data and when. The log captures the user, patient, type of data accessed, timestamp, and purpose (if break-glass was used). The DPO can review this log and generate access reports for data subject requests.

9.3 Consents & DSR

Patient consents (Compliance > Consents) are recorded and tracked in the system. Each consent captures the patient, the type of consent (treatment, data processing, research, etc.), the date given, and the consenting party. Consents can be created, updated, and revoked. The system enforces that certain procedures cannot proceed without valid consent on file.

Data Subject Requests (Compliance > DSR) allow patients to exercise their rights under the Data Privacy Act — to access, correct, or delete their personal data. The DPO receives a DSR, verifies the patient's identity, and processes the request through a defined workflow: receive → verify → start → complete, or reject. Each step is logged with timestamps and the DPO's notes. The system tracks SLA deadlines for DSR completion.

10. Reports & Exports

The Reports module (Reports > Dashboard) provides a comprehensive set of operational and regulatory reports across all modules. Each report can be filtered by date range, branch, and other parameters, and exported to CSV or XLSX. Key reports include: Patient Census (daily/monthly patient counts), Laboratory TAT (turnaround time analysis), Pharmacy Dispensing Summary, Stock Movement Ledger, Accounts Receivable Aging, Cashier Shift Summary, Controlled Drug Register, and Audit Trail Export. Reports are accessible based on the user's role and permissions. To run a report, select it from the catalog, set your filters, and click "Run". To export, click the "Export" button and choose CSV or XLSX.

11. Frequently Asked Questions (Quick Guide)

11.1 Login & Access

Q: I forgot my password. What do I do?
A: Contact your tenant administrator. They can reset your password from the Users screen. You will receive a temporary password that you must change on first login.

Q: Why can't I see a module in the navigation?
A: Your role may not have permission for that module. Contact your tenant administrator to review your role assignments and permissions.

Q: What is the tenant slug?
A: The tenant slug is a short identifier for your organization. It is assigned when your tenant is created and is required at login. If you don't know it, ask your tenant administrator.

11.2 Patients & Clinical

Q: How do I register a walk-in patient?
A: Go to Patients, click "New Patient", fill in the minimum required fields (name, date of birth, sex), and save. The system generates an MRN automatically.

Q: Can I edit a patient's name after registration?
A: Yes, but the change is logged in the audit trail. Open the patient record, click Edit, make the correction, and save.

Q: How do I merge duplicate patient records?
A: A user with patient.merge permission can open one record, select "Merge", and choose the duplicate record to merge into it. The merged record is archived and all data is consolidated.

Q: Why can't I sign a clinical note?
A: You may not have the clinical.note.sign permission, or the note may already be signed. Signed notes are locked; corrections require an amendment.

11.3 Laboratory

Q: Why can't I verify a lab result I entered?
A: The system enforces segregation of duties (AC-17). The person who enters a result cannot verify it. A different user with the lab_verifier role must perform the verification.

Q: What do the result flags (N, L, H, LL, HH) mean?
A: N = Normal, L = Low, H = High, LL = Critical Low, HH = Critical High. Critical values trigger an alert that must be acknowledged.

Q: How do I release a lab report?
A: All results on the order must be verified first. Then a user with lab.report.release permission can click "Release Official Report" on the order. The report is generated as a PDF by the background job system.

Q: What happens if a specimen is rejected?
A: The specimen is marked as rejected with a reason (e.g. hemolyzed, clotted, insufficient volume). The ordering doctor is notified and may need to order a recollect.

11.4 Pharmacy

Q: How do I dispense a prescription?
A: After the pharmacist verifies the prescription, click "Verify & Dispense". The system auto-selects the batch with the earliest expiry date (FEFO). Review the allocation, click "Dispense", and the system prints a label and updates stock.

Q: Why is the system suggesting a specific batch for dispensing?
A: The system uses FEFO (First-Expired, First-Out) to select the batch with the earliest expiry date, minimizing waste. You can override the suggestion but must provide a reason.

Q: What is a controlled drug?
A: A controlled substance (dangerous drug under RA 9165) that requires special prescription handling, no refills, dispensing/record-keeping, and periodic reporting. Only users with the pharmacy.controlled permission can handle these.

Q: Can I dispense a prescription without verification?
A: No. All prescriptions must be verified by a pharmacist before dispensing. This is a mandatory safety check.

11.5 Central Supply

Q: Why can't I approve a purchase order I created?
A: The system enforces segregation of duties (G-11). The person who creates a PO cannot approve it. A different user with the supply_approver role must perform the approval.

Q: What is a 3-way match?

A: A 3-way match verifies that the purchase order (what was ordered), the GRN (what was received), and the supplier invoice (what was billed) all agree on quantities and prices. Only when all three match can the invoice be approved for payment.

Q: How do I handle a stock discrepancy found during a count?
A: Create a stock adjustment with the variance and a reason. The adjustment goes through the approval chain (supply_approver approves, inventory_controller posts). The adjustment updates the stock balance and creates an audit trail entry.

Q: What is the difference between a requisition and a purchase order?
A: A requisition is an internal request from a department to the central store. A purchase order is an external order to a supplier for stock replenishment.

11.6 Billing & Cashiering

Q: How do I process a payment?
A: Go to Billing > POS, search for the patient, select the invoice, choose the payment method (cash, card, insurance), enter the amount tendered, and click "Complete Payment". The system prints an Official Receipt.

Q: How do I handle a refund?
A: Create a credit note against the original invoice with the refund amount and reason. The credit note requires approval. Once approved, the refund can be processed. The credit note links back to the original payment for audit.

Q: What happens if the cash drawer doesn't match at shift close?
A: The system shows the variance (over or short). Enter the reason for the discrepancy and confirm. The variance is recorded in the shift report and visible to the branch manager.

Q: What are split payments?
A: The POS supports paying an invoice with multiple payment methods (e.g. partial cash + partial card). Add each payment line separately until the balance is zero, then click "Complete Payment".

11.7 Keyboard Shortcuts

Ctrl+K — Global search (patients, orders, invoices, items)
Enter — Submit form / confirm dialog
Escape — Close dialog / cancel action
Tab — Move to next field
Shift+Tab — Move to previous field
Ctrl+S — Save (on supported forms)

Glossary

Term	Definition

AC-17	A policy requiring that the person who performs a step (e.g. result entry) must be different from the person who performs the next step (e.g. result verification) — segregation of duties in the lab
FEFO	First-Expired, First-Out — the pharmacy dispensing logic that selects the batch with the earliest expiry date to minimize waste
3-Way Match	Matching the purchase order, goods receipt note, and supplier invoice to verify quantities and prices agree before approving payment
MRN	Medical Record Number — the unique patient identifier generated automatically on registration
OR	Official Receipt — the numbered document generated when a patient makes a payment at the POS
GRN	Goods Receipt Note — the document created when stock is received against a purchase order
DSR	Data Subject Request — a request from a patient to access, correct, or delete their personal data under the Data Privacy Act
PHI	Protected Health Information — any patient health data protected under the Data Privacy Act
SOX	Segregation of Duties — a security policy requiring that no single user can perform conflicting actions in a transaction chain
TAT	Turnaround Time — the time elapsed from test ordering to result reporting/report release
Break-Glass	An emergency override that grants temporary access to restricted data, logged with a justification and time limit
Tenant	A single organization using the clinic platform, fully isolated from other tenants at the database level (RLS)
DPO	Data Protection Officer — the person responsible for data privacy compliance within the organization
LOINC	Laboratory LOINC Codes — the international standard for identifying laboratory and clinical observations